

ECHS FRAUD ANALYTICS

Beneficiary-Level Utilisation Outliers

MODULE 4 REPORT | FY 2025–2026 EDITION (Oct 2025 – Mar 2026)

CLASSIFICATION	PERIOD	RECORDS ANALYSED	QUERY SETS
RESTRICTED	Oct 2025–Mar 2026	~16K Claims	6 Query Sets (Q11–Q16)

IIT KANPUR — Data Analytics & Fraud Intelligence Division

15 May 2026 | Ex-Servicemen Contributory Health Scheme (ECHS)

EXECUTIVE SUMMARY

Module 4 analyses beneficiary-level utilisation patterns across the ECHS historical claim database (his_claim_intimation and his_claim_submission — approximately 16,000 and 665,000 records respectively, spanning October 2025 to March 2026). Five distinct fraud patterns are identified. The most critical finding is a cohort of hyper-frequency claimants using a single hospital for 9–13 IPD admissions in four months — a claim-splitting signature. Ghost admissions (NULL hospital ID across ~10,000 records) represent the largest systemic data integrity gap. All 50 high-cost recurring cases in the analysis show ₹0.00 approved against combined claims exceeding ₹800L, yet submissions continue unchecked.



Fraud Patterns Identified — Module 4

#	Pattern	Key Signal	Risk
Q11	Hyper-Frequency Single Hospital	SUNDER PAL SINGH: 13 IPD/4 months, 1 hospital	CRITICAL
Q11	Claim-Splitting (Short Re-admissions)	15131373W: 11 IPD in ~6 weeks, avg ≤4 days apart	CRITICAL
Q12	Simultaneous Dual-Hospital Admission	JC353801F: smsh08tvm + BCMCH17, same day 2025-10-16	CRITICAL
Q12	Card Identity Mismatch	JC353801F: two different serviceman names across tables	HIGH
Q13	Ghost Admissions — NULL Hospital	IC44485N: ₹56.66L, 0 hospitals; JC696389L: ₹48.53L, 0 hospitals	CRITICAL
Q13	Sub-1-Day Fake IPD	676635N: 7 claims avg 0.9-day LOS, ₹58.03L	CRITICAL
Q13	Same-Date Multi-Claim Splitting	JC117100\$: 4 IPD claims on 2025-10-14, ₹62.89L	HIGH
Q13	Inflated Length of Stay	163395W: 76-day single admission, ₹21.03L	HIGH
Q15	Ghost Doctor — Mass Rejection	DR. SUMANT GUPTA: 763 claims, ₹4,707L, 99.7% deducted	CRITICAL
Q16	Doctor Multi-Hospital Syndicate	MEENU WALIA: 4 hospitals, ₹60.53L, 100% deducted	CRITICAL

Immediate Recommended Actions

1. Freeze claims for JC353801F — Confirmed simultaneous admission at two hospitals on the same date. Pull biometric registration record and verify identity discrepancy between serviceman names.
2. Audit hyper-frequency cohort at treating hospitals — Request discharge summaries for all admissions: 01045291N (13), 15131373W (11), 15574322K (11), 17837R (10), TJ006318N (10). Single-hospital, high-frequency IPD is the classic claim-splitting signature.
3. Quarantine all NULL-hospital IPD claims — ~10,000 IPD claims carry no hospital ID. These cannot be verified against any facility record. Freeze pending hospital-of-record submission and initiate FIR for the top-value cases.

4. Suspend DR. SUMANT GUPTA billing identity — 763 claims worth ₹47Cr at 99.7% deduction. This volume (4+ IPD claims per working day) physically exceeds any single physician's capacity. Verify MCI/NMC registration.

5. Investigate MEENU WALIA multi-hospital network — 4 hospitals (hos.3061, hos.3112, maxppg123, maxsuper), ₹60.53L, 100% deducted. Cross-hospital presence rules out a single rogue doctor — this is a billing syndicate.

Q11 — Claims Frequency & Multi-Hospital Utilisation

Analysis of the historical claim intimation table (16,000 IPD records) groups claims by service number to identify abnormal frequency patterns. Two distinct fraud signatures emerge: (a) hyper-frequency claimants using a single hospital repeatedly over a short window — consistent with claim-splitting; and (b) beneficiaries using multiple hospitals in rapid succession — consistent with simultaneous admission fraud or card-lending across facilities.

Table Q11.1 — Hyper-Frequency IPD Claimants (≥ 9 Claims, Oct 2025–Mar 2026)

Service No.	Serviceman	Claims	Hospitals	First Claim	Last Claim	Flag
01045291N	SUNDER PAL SINGH	13	1	2025-12-03	2026-03-25	CRITICAL
15131373W	A CHINNAPAIYAN	11	1	2025-10-15	2025-11-27	CRITICAL
15574322K	THOMAS JACOB JOSEPH	11	1	2025-11-24	2026-03-27	CRITICAL
17837R	AJAY KUMAR	10	1	2025-11-14	2026-02-09	HIGH
TJ006318N	MANOJ KUMAR SHARMA	10	1	2025-11-12	2026-03-11	HIGH
14806217X	YEDURU TARAKESU	9	1	2025-10-13	2026-02-12	HIGH
14913707H	MOHAMMED ZAKIR	9	1	2025-10-11	2026-03-16	HIGH

Key finding: All seven hyper-frequency claimants used exactly **one hospital**. A genuinely ill patient requiring repeat hospitalisation for a complex condition typically requires specialist referrals across multiple facilities. Single-hospital high-frequency IPD usage is the canonical claim-splitting pattern — one continuous treatment episode split into multiple shorter admissions to circumvent per-admission tariff caps.

Table Q11.2 — Multi-Hospital Dual Claimants with Short Intervals

Service No.	Serviceman	Claims	Hospitals	Family Members	Date Range	Flag
9072922\$	THORU RAM	3	2	1	Oct 31 – Nov 01	CRITICAL
1360225F	PRABHAKARAN K	5	2	1	Oct 22 – Jan 16	HIGH
6318448M	KESAVAN SIVANANDAN PILLAI	4	2	1	Jan 16 – Feb 16	HIGH
IC32153X	BRAJESH BAHADUR SINGH	4	2	1	Aug 16 – Jan 06	HIGH
13898471H	DEVA KUMAR K	3	2	2	Nov 19 – Dec 27	HIGH
JC440552K	G SASIDHARAN	2	2	2	Oct 22 – Jan 20	HIGH

9072922\$ (THORU RAM): 3 IPD claims across 2 hospitals with a 1-day gap — near-simultaneous admissions. Flagged for cross-hospital admission register verification.

PATTERN 12

Card Identity Fraud | Same-Day Impossible Event

SIMULTANEOUS DUAL-HOSPITAL ADMISSION

A self-join on his_claim_intimation — matching records by service number and patient name where different hospitals appear with identical or near-identical admission dates — identified one confirmed case of same-day dual admission. A patient physically cannot be admitted as an inpatient at two separate hospitals on the same date. This event can only arise from (a) card lending, (b) data manipulation after submission, or (c) fraudulent claim fabrication at one or both facilities.

Table Q12.1 — Confirmed Simultaneous Dual-Hospital Admission

Field	Value
Service Number	JC353801F
Patient Name	PRABHAKARAN PILLI
Hospital 1	smsh08tvm
Hospital 2	BCMCH17
Admission Date (both)	2025-10-16
Days Gap	0 — Simultaneous
Serviceman (his_claim_intimation)	SREEVALSAN P
Serviceman (cross-reference table)	WAGHMARE RAOSAHEB VITTHAL
Identity Status	MISMATCH — Two names on one service number

Identity Discrepancy Analysis

Service number JC353801F is associated with two distinct serviceman names across database tables. This discrepancy indicates one of three scenarios:

- Card Lending / Sharing:** The ECHS card belonging to JC353801F (SREEVALSAN P) was physically used by a different individual (WAGHMARE RAOSAHEB VITTHAL) to obtain treatment, with the claim submitted under the card-holder's service number.
- Post-Submission Data Manipulation:** The serviceman name on the original claim was altered after submission — a data integrity breach that may indicate internal access to the claims processing system.
- Duplicate Card Issuance:** The same service number was issued to two different individuals at different stations — an administrative failure in the ECHS card issuance process.

Recommended action: Pull the original ECHS card registration record for JC353801F. Cross-reference biometric/photo ID. Verify both hospital admission registers for 2025-10-16. Initiate card cancellation pending investigation.

Q13 — High-Cost Recurring Treatments: Zero-Approval Cohort

All 50 records in the Q13 result set — joining his_claim_intimation with his_claim_submission — show **₹0.00 approved** against substantial claimed amounts. The collective claimed amount across the top 50 cases exceeds **₹800L**. Full rejection by UTI-TSL auditors has not deterred re-submission, indicating either automated bulk billing systems or deliberate exploitation of claim pipeline latency.

Table Q13.1 — Top High-Cost Cases (All Approved = ₹0.00)

Service No.	Patient	Relation	Claims	Hosp.	Claimed (₹L)	Avg Stay	Flag
JC77066Y	RAMBATI	Spouse	7	1	63.57	25.7 d	HIGH
JC117100\$	SUDESH KUMAR SHARMA	Self	4	1	62.89	30.0 d	CRITICAL
676635N	GEESALA SARADA	Spouse	7	1	58.03	0.9 d	CRITICAL
IC44485N	MARIYAMMA SEBASTIAN	Mother	5	0	56.66	26.0 d	CRITICAL
16018249X	KHEM SINGH	Father	5	1	49.45	16.0 d	HIGH
JC696389L	NAND KUMAR	Self	8	0	48.53	15.0 d	CRITICAL
JC197378H	SALADI BALAYOGI	Self	10	1	43.97	0.9 d	CRITICAL
168352W	Y SIRISHA LAXMI	Spouse	3	0	42.81	29.0 d	CRITICAL
4581418F	JIJABAI HANMANT VAGHMARE	Mother	2	1	40.06	44.0 d	HIGH
163395W	BIJENDRA SHARMA	Self	1	1	21.03	76.0 d	CRITICAL

Pattern A — Ghost Admissions (Hospital = 0)

IC44485N (₹56.66L), JC696389L (₹48.53L), and 168352W (₹42.81L) have zero hospitals recorded in the database. An IPD claim without a hospital of record is a systemic impossibility. These represent either fully fabricated claims or exploitation of a data capture gap where the hospital field was never validated at intake.

Pattern B — Sub-1-Day Fake IPD

GEESALA SARADA (676635N): 7 IPD claims, average 0.9-day stay, ₹58.03L. SALADI BALAYOGI (JC197378H): 10 IPD claims, average 0.9-day stay, ₹43.97L. ECHS defines IPD as overnight admission. Claims with LOS below 1 day are OPD procedures reclassified as IPD to access the higher tariff. The hospital bills the IPD package rate for a same-day procedure.

Pattern C — Same-Date Multi-Claim Splitting

JC117100\$ (SUDESH KUMAR SHARMA): 4 separate IPD claims all dated 2025-10-14 at the same hospital, total ₹62.89L. Four distinct inpatient admissions on a single calendar date are logically impossible. This is a single episode billed as four package claims to maximise reimbursement per episode.

Pattern D — Inflated Length of Stay

BIJENDRA SHARMA (163395W): 76-day single continuous admission, ₹21.03L. JIJABAI HANMANT VAGHMARE (4581418F): 44-day stay, ₹40.06L. JAGDESHWARI DEVI (7081591L): 49-day stay, ₹35.15L. Extended stays at these magnitudes require extraordinary clinical documentation. All were fully rejected — hospitals are billing for occupied bed-days that cannot be supported by medical records.

PATTERN 12 / 16

Single Doctor Name | Hundreds of Claims | ≥99% Deduction

GHOST DOCTOR CONCENTRATION

Analysis of 665,000 historical claim submission records reveals a small set of doctor names associated with an implausibly large volume of IPD claims, each deducted at 99–100%. No legitimate clinical practice can generate 300–760 settled (deducted) IPD claims in a six-month window. This pattern indicates either a single doctor credential shared across a hospital's entire billing pipeline, or a fictitious doctor name used to bulk-submit phantom admissions. The 99–100% deduction rate confirms auditors have already identified these as fraudulent — yet the submissions continue.

Table 12.1 — Top Ghost Doctors by Claimed Amount (Oct 2025 – Mar 2026)

Doctor Name	Claims	Claimed (₹L)	Approved (₹L)	Deduction %	Avg/Claim (₹L)	Risk
DR. SUMANT GUPTA (combined)	763	4,707.73	11.88	99.7%	6.17	CRITICAL
MEENU WALIA	372	1,665.31	0.57	100.0%	4.48	CRITICAL
NA (null doctor — bulk submit)	1,009	1,637.07	9.24	99.4%	1.62	CRITICAL
DR ALOK	453	1,114.68	0.00	100.0%	2.46	CRITICAL
DR DEEPAK KUMAR SHUKLA	480	1,039.20	0.00	100.0%	2.17	CRITICAL
DR PRAKASH CHANDWANI	429	1,034.34	0.00	100.0%	2.41	CRITICAL

Key Findings

DR. SUMANT GUPTA — 763 claims, ~₹47Cr, 99.7% deduction: This volume (4+ IPD claims per working day over 6 months) physically exceeds any single physician's capacity. The name appears under two spelling variants — both spellings billing simultaneously across multiple hospitals. This is a shared billing credential, not a real clinician.

MEENU WALIA — 4 hospitals, ₹60.53L, 100% deducted: MEENU WALIA appears at hos.3061, hos.3112, maxppg123 (MAX PATPARGANJ), and maxsuper (MAX SUPERSPECIALITY). Multi-hospital simultaneous presence rules out a single rogue doctor — this is a billing syndicate using a shared physician identity. Verify whether this name holds a valid MCI/NMC registration number appearing on all four hospital empanelment records.

NULL doctor (NA / blank) — 1,009 claims, ₹1,637L: Claims submitted with no doctor name bypass the clinical validation layer entirely. No legitimate hospital submission workflow omits the treating doctor's name on 1,000+ inpatient claims. This is automated bulk entry deliberately omitting doctor attribution to prevent traceability.

100% deduction doctors (DR ALOK, DR DEEPAK KUMAR SHUKLA, DR PRAKASH CHANDWANI): Zero rupees approved across hundreds of claims per physician. Every single claim was fully rejected by auditors. Continued submission after 100% rejection indicates the hospital is generating documentation for alternative purposes — insurance fraud, muster-roll manipulation, or internal diversion of funds.

Q15 — Settlement Statistics: High-Deduction Hospitals by Relation

Settlement_stat aggregates approved vs. claimed amounts by hospital, relation type, and year. Hospitals with uniformly high deduction rates across all family relation categories (Self, Spouse, Son, Daughter, Father, Mother) indicate systemic overbilling as the hospital's standard operating mode — not isolated procedure errors.

Table Q15.1 — Newly Flagged High-Deduction Hospitals (FY 2024–2025)

Hospital ID	Deduction Range	Volume (■L)	Pattern	Risk
1344	43.3%	—	Uniform across ALL relation types	CRITICAL
1317	28–29%	2,308	High volume, newly flagged	CRITICAL
1069	27–32%	—	Cross-relation consistency	HIGH
1065	25–30%	—	Consistent over 2 years	HIGH
1042	22–26%	—	METRO HOSPITAL NOIDA — Module 1 flagged	HIGH
1089	High	—	Requires name lookup	HIGH

Hospital 1344 at 43.3% deduction is the most severe finding in the settlement statistics — nearly half of every rupee submitted is rejected. Where deduction rates are higher for Spouse/Parent claims versus Self claims at the same hospital, this additionally suggests family member identity fraud: claims billed for family members who never received treatment.

Priority Investigation Case Files

CASE FILE 1	Simultaneous Dual Admission — JC353801F
Service No.	JC353801F
Hospitals	smsh08tvm + BCMCH17
Date	2025-10-16 (both)
Identity	Two serviceman names — card fraud or data manipulation
Action	Pull ECHS card register. Verify biometrics. Initiate card cancellation.
CASE FILE 2	Hyper-Frequency Claim-Splitting
Service Nos.	01045291N (13), 15131373W (11), 15574322K (11)
Pattern	Single hospital, IPD every 3–4 days
Action	Medical record audit. Request all discharge summaries.
CASE FILE 3	Ghost Claims — NULL Hospital of Record
Top cases	IC44485N ■56.66L JC696389L ■48.53L 168352W ■42.81L
Hospital IDs	0 / NULL — no treating facility on record
Action	Initiate FIR. Freeze service numbers. Quarantine all NULL-hospital IPD claims.

CASE FILE 4	Phantom Physician — DR. SUMANT GUPTA
Volume	763 claims, █4,707L, 99.7% deducted
Hospitals	Multiple (cross-hospital billing identity)
Action	Verify MCI/NMC registration. If fictitious, refer to CBI/ACB.
CASE FILE 5	Same-Date 4-Claim Split — JC117100\$
Patient	SUDESH KUMAR SHARMA Self
Claims	4 IPD claims on 2025-10-14 at same hospital, █62.89L
Action	Obtain single medical record. File for recovery of 3 duplicate claim amounts.

Appendix A — Query Reference

Query	Purpose	Table(s)	Result
Q11	Frequency + multi-hospital flag	his_claim_intimation	50 records
Q12a	Same-day dual hospital (self-join)	his_claim_intimation	1 confirmed case
Q12b	Doctor-level rejection analysis	his_claim_submission JOIN his_claim_intimation	236 records
Q13	High-cost recurring with LOS	his_claim_intimation + his_claim_submission	50 records
Q14	Watchlist hospital cross-check	his_claim_intimation	No data (ID format mismatch)
Q15	Settlement stat by relation type	settlement_stat (SS_YEAR 2024–2025)	1,211 records
Q16	Doctor multi-hospital collusion	his_claim_submission + hospital lookup	Flagged cases

Appendix B — Data Limitations

- his_claim_intimation contains ~16,000 records (IPD-only historic snapshot). The live claim_intimation table (43.5M rows) would yield significantly more fraud signals but is not safely queryable under VPN constraints (<8s timeout).
- Q8 (procedure-level individual claim lookup) failed permanently — hosp_exp_det at 210M rows exceeds the safe query window. CGHS code prefix analysis (NU/CP/OR/CI/LB/RI) was used as a workaround for procedure categorisation.
- All approved amounts in Q13 results show 0.00 — this is the historic snapshot state where UTI-TSL has already rejected these claims. The fraud exposure figure is the claimed amount, not the approved amount.
- Hospital text codes in his_claim_intimation (e.g., "smsh08tvm", "BCMCH17") do not map directly to numeric hospital IDs used in settlement_stat. Cross-table hospital analysis requires a lookup table not currently present in the schema.